

Cutaneous Lesions

Mammary Paget disease (MPD) typically presents as a unilateral, erythematous, scaly plaque or patch involving the nipple and occasionally the areola. The lesion often has an eczematous appearance with ulceration, weeping, erosion, and nipple discharge. Nipple retraction may occur. Patients frequently report pain, burning, and pruritus.

Extramammary Paget disease (EMPD) clinically resembles MPD and commonly appears as a well-demarcated, moist, erythematous, scaly, eczematous patch. Hypopigmentation or hyperpigmentation may be present. Burning and intense pruritus are common symptoms. EMPD primarily affects apocrine gland-rich areas, most frequently the vulva, followed by the perineal, scrotal, perianal, and penile regions. Rare cases of ectopic EMPD have been reported in areas with few or no apocrine glands, such as the chest, abdomen, thigh, eyelids, face, and external auditory canal.

Both MPD and EMPD are frequently misdiagnosed as inflammatory or infectious dermatitis, leading to delays in diagnosis—often ranging from months to years. Initial treatment with topical corticosteroids or antifungal agents for presumed eczema or infection is common. A definitive diagnosis is usually made only after failure to respond to these therapies, prompting a biopsy.

Related Physical Findings

Approximately 50% of patients with MPD have a palpable underlying breast mass. Among those with a detectable tumor, half will have axillary lymphadenopathy due to metastatic spread. In contrast, palpable lymph nodes are uncommon in EMPD, and an underlying malignancy is less frequently associated, although it may be present in a minority of cases.

A thorough physical examination is essential in all patients with MPD or EMPD to evaluate for signs of underlying malignancy or metastatic disease. Symptoms and findings related to an associated carcinoma may also be present at presentation.

Laboratory Tests

Pathology

MPD and EMPD are intraepidermal adenocarcinomas with similar histologic features. Characteristic findings include the presence of large, atypical cells singly, in clusters, or in sheets within the epidermis. These cells exhibit nuclear enlargement, nuclear atypia, prominent nucleoli, and abundant, well-defined cytoplasm (Fig. 121-4). Intercellular bridges are absent.

The Paget cells may occupy all levels of the epidermis and can extend into hair follicles and sweat gland ducts. They compress but do not disrupt the basal layer, and there is no formation of junctional nests. Epidermal changes such as acanthosis, hyperkeratosis, and parakeratosis are commonly seen.

These cells display a "pagetoid" spread pattern and must be differentiated from other intraepidermal malignancies, including: - Melanoma

- Pagetoid squamous cell carcinoma in situ
- Mycosis fungoides
- Cutaneous adnexal carcinomas (e.g., sebaceous carcinoma, porocarcinoma)
- Merkel cell carcinoma
- Langerhans cell histiocytosis
- Other epidermotropic metastatic carcinomas